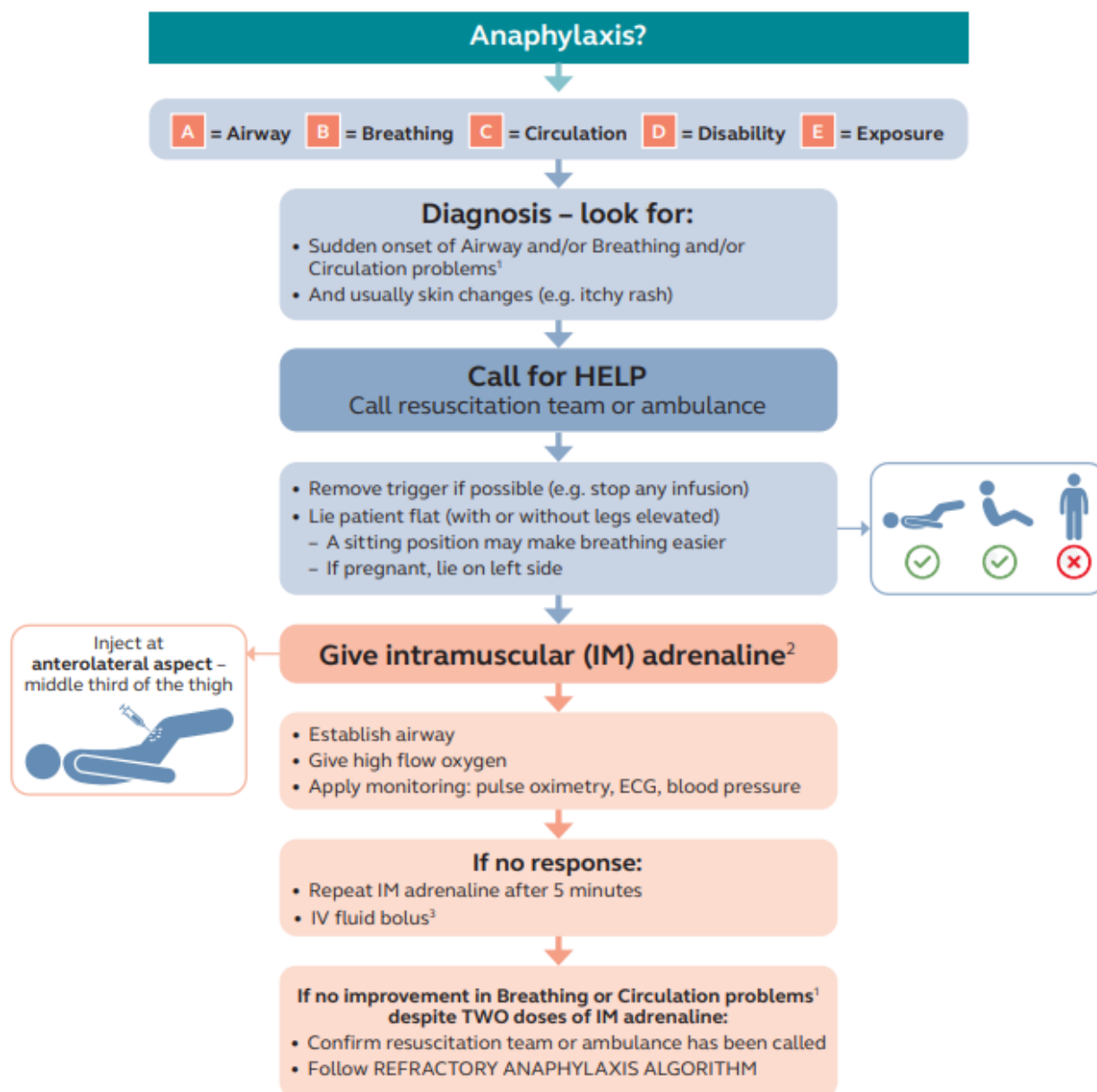


Anaphylaxis



1. Life-threatening problems

Airway

Hoarse voice, stridor

Breathing

↑work of breathing, wheeze, fatigue, cyanosis, SpO₂ <94%

Circulation

Low blood pressure, signs of shock, confusion, reduced consciousness

2. Intramuscular (IM) adrenaline

Use adrenaline at 1 mg/mL (1:1000) concentration

Adult and child >12 years: 500 micrograms IM (0.5 mL)

Child 6–12 years: 300 micrograms IM (0.3 mL)

Child 6 months to 6 years: 150 micrograms IM (0.15 mL)

Child <6 months: 100–150 micrograms IM (0.1–0.15 mL)

The above doses are for IM injection **only**.

Intravenous adrenaline for anaphylaxis to be given **only by experienced specialists** in an appropriate setting.

3. IV fluid challenge

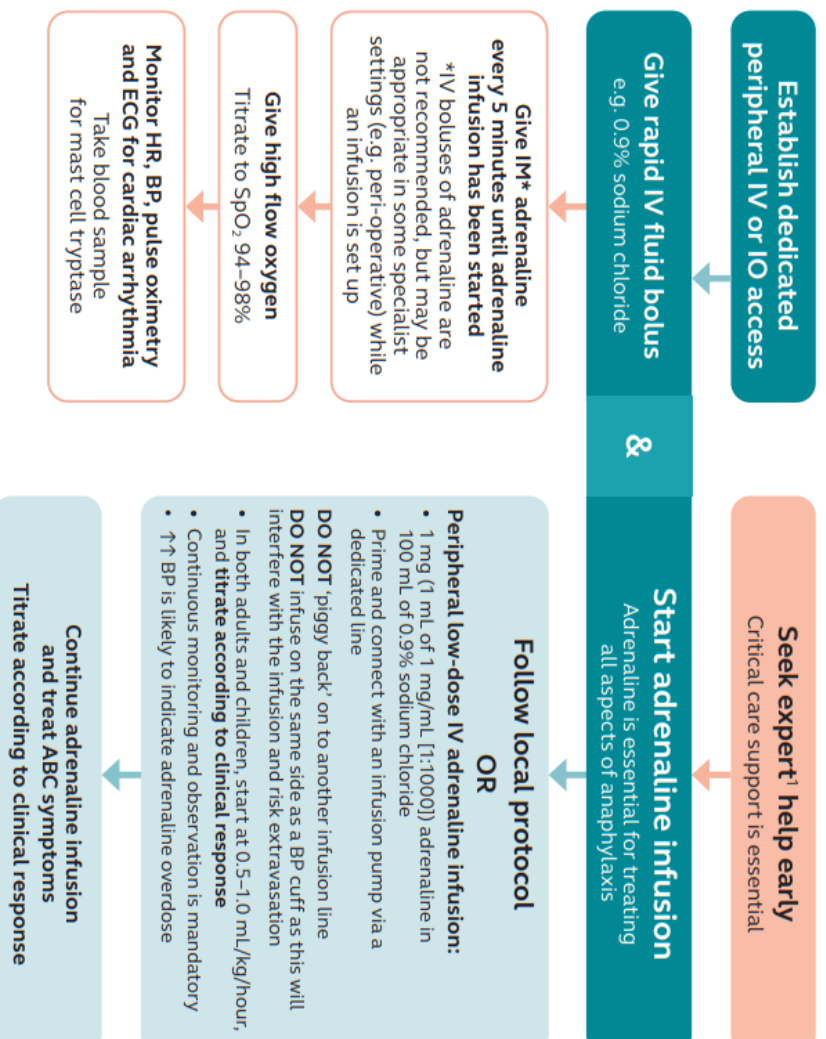
Use crystalloid

Adults: 500–1000 mL

Children: 10 mL/kg

Refractory anaphylaxis

No improvement in respiratory or cardiovascular symptoms despite 2 appropriate doses of intramuscular adrenaline



A = Airway

Partial upper airway obstruction/stridor:

Nebulised adrenaline (5mL of 1mg/mL)

Total upper airway obstruction:

Expert help needed, follow difficult airway algorithm

B = Breathing

Oxygenation is more important than intubation

If apnoeic:

- Bag mask ventilation
- Consider tracheal intubation

Severe/persistent bronchospasm:

- Nebulised salbutamol and ipratropium with oxygen
- Consider IV bolus and/or infusion of salbutamol or aminophylline
- Inhalational anaesthesia

C = Circulation

Give further fluid boluses and titrate to response:

Child 10 mL/kg per bolus

Adult 500–1000 mL per bolus

- Use glucose-free crystalloid

(e.g. Hartmann's Solution, Plasma-Lyte®)

Large volumes may be required (e.g. 3–5 L in adults)

Place arterial cannula for continuous BP monitoring

Establish central venous access

IF REFRACTORY TO ADRENALINE INFUSION

Consider adding a second vasopressor in addition to adrenaline infusion:

- Noradrenaline, vasopressin or metaraminol
- In patients on beta-blockers, consider glucagon

Consider extracorporeal life support

Cardiac arrest – follow ALS ALGORITHM

- Start chest compressions early
- Use IV or IO adrenaline bolus (cardiac arrest protocol)
- Aggressive fluid resuscitation
- Consider prolonged resuscitation/extracorporeal CPR

†Intravenous adrenaline for anaphylaxis to be given only by experienced specialists in an appropriate setting.